

Septidin 10% Solution

Composition: Contains Povidone Iodine 10% w/v
Description: Reddish brown solution with odor of iodine
Indication: Antiseptic Against Bacteria, Fungi, Viruses, Protozoa, Cysts & Spores For Minor Burns, Wounds & On Pre-Operative Preparation Of The Skin

Direction of Use:
Use full strength. Do not dilute.
Pre-operation preparation of the skin: Wet the operation site with water. Apply SEPTIDIN 10%, work up a lather using sterile gauze. Scrub site for 5 minutes, rinse off with water using wet sterile gauze. Paint the surgical site with SEPTIDIN 10% using sterile gauze and permit to air dry.
Minor burns, wounds skin infections: Apply twice daily to affected area with clean gauze or cotton bud.

Contraindication:
SEPTIDIN 10% solution is contraindicated in patients with known hypersensitivity to iodine or to any of the ingredients in the product and Neonates.

Warning and precaution:
For external use only.
Local irritation or hypersensitivity reactions may occur. Discontinue use immediately if symptoms develop.
Povidone-iodine is not recommended for regular use in children below 2 years and neonates and is contra-indicated in very low birth weight infants (below 1500g). It is must not be used in deep wounds.
Avoid applying the medication to severe burns or to large areas otherwise denuded of skin. Avoid regular use in pregnancy, when breastfeeding, and in patients with thyroid disorders. Prolonged exposure to wet solution may cause irritation or rarely, severe skin reaction.
Regular or prolonged use should be avoided in patients with:

- 1. Burns, especially more than 20% of body surface area.
- 2. Large open wounds
- 3. Lithium therapy
- 4. Renal failure
- 5. Thyroid disease

Drug interaction:
Povidone iodine can be expected to react with protein and various other organic substances, e.g. blood and pus constituents, which may impair its efficacy. When povidone iodine co-administered with enzymatic wound treatment agents, the enzyme component is oxidised by the iodine, weakening the effect of both products. A reciprocal weakening of effect also occurs with concomitant use of povidone iodine and silver containing disinfectants, hydrogen peroxide, and taurolidine. Povidone iodine must not be used at the same time or shortly after mercury-containing wound treatments or disinfectants because, in certain circumstances, iodine and mercury can produce a substance that damages the skin. Patients receiving concomitant lithium treatment should avoid regular, in particular extensive because the iodine taken up may promote the triggering of an under-active thyroid gland, which is possible with lithium. Povidone iodine is also incompatible with reducing substances, alkaloid salts, tannic acid, salicylic acid, silver salt, mercury salt and bismuthate.

Side effects:
It is irritant to the skin, could also cause hypersensitivity reactions, which may include urticaria, angioedema, cutaneous haemorrhage or purpuras, fever, arthralgia, lymphadenopathy, and eosinophilia. Relatively long-term use on extensive wound and burn areas can cause noteworthy iodine uptake. In isolated cases, patients with a history thyroid diseases may develop an over-active

thyroid gland (iodine-induced hyperthyreosis), in some cases with symptoms such as an accelerated pulse or anxiety. Reports of (additional) electrolyte and serum osmolality disturbances, as well as impaired kidney function and blood hyperacidity (metabolic acidosis), have been described after the use of relatively large quantities of povidone iodine-containing drugs (e.g. in the treatment of burns).

Overdosage:
Symptoms of overdosage: After accidental oral ingestion of large quantities of Povidone iodine, symptoms of acute iodine intoxication can occur; these include abdominal pain, nausea, vomiting, diarrhoea, dehydration, hypotension with a (persistent) tendency to collapse, glottideal oedema, tendency to bleed (mucous membranes, kidneys), cyanosis, renal damage (globular and tubular necrosis) leading to anuria (after 1-3 days), paraesthesia, fever and pulmonary oedema. With long term excessive iodine intake, symptoms of hyperthyreosis-tachycardia, anxiety and tremor-may occur.
Management of overdosage: Immediate administration of foods containing starch and protein, e.g. starch flour stirred into milk or water, gastric lavage with 5% sodium thiosulphate solution, or a starch suspension. If already absorbed, toxic serum iodine levels can be effectively lowered by peritoneal dialysis or haemodialysis. Thyroid function should be carefully monitored to exclude or detect early any iodine-induced hyperthyreosis. Further treatment depends on the presence of any other symptoms, e.g. metabolic acidosis and renal insufficiency. Treatment of iodine-induced hyperthyreosis iodine-induced hyperthyreosis is treated according to the form of the disease. In some circumstances, mild forms may require no treatment; marked forms thyreostatic treatment (which is of only delayed efficacy), and the most severe forms (thyreotoxic crisis) intensive treatment, plasmapheresis or thyroidectomy.
Topical application of Iodine Solution can stain the skin to a deep reddish-brown colour. This can be removed using dilute solutions of alkalis or Sodium Thiosulphate. In case of contact with eyes, flush with water for 15 minutes. If irritation persists, contact a doctor. Seek professional advice or contact a poison center immediately.
Symptoms of acute poisoning would include metallic taste, vomiting, abdominal pain, and diarrhoea. Victims could be given a copious draughts of milk and starch mucilage; lavage may be attempted if there is no oesophageal damage. Other treatments include Activated Charcoal and Sodium Thiosulphate Solution 1% or 5%.

Storage:
Store below 30°C. Protect from light.

Presentation:
Bottles of 50 ml, 100 ml, 500 ml, 1 litre and 4 litres.

Manufactured For:
Steriline Sdn. Bhd. Registration No. 199401039241 (324926-X)
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